

CHAPTER ONE

1.1 About SIWES

The Student Industrial Work Experience Scheme (SIWES) is a skill acquisition and practical training program initiated by the Federal Government of Nigeria through the Industrial Training Fund (ITF) in 1973. It was established to bridge the gap between theoretical classroom learning and real-life industrial practice for students pursuing studies in science, technology, engineering, agriculture, and environmental disciplines. Before its introduction, Nigerian students faced significant challenges in gaining hands-on industrial experience, leading to a disconnect between academic knowledge and workplace demands.

The primary objective of SIWES is to expose students to real work environments, enabling them to apply theoretical concepts to practical situations. Through participation in the scheme, students develop relevant technical and professional skills, enhance their problem-solving abilities, and understand the dynamics of industrial operations. It provides students with an opportunity to interact with experienced professionals, learn modern work ethics, and appreciate the importance of discipline, teamwork, and innovation in the workplace.

The program typically lasts between three to six months and is undertaken during or after the penultimate year of study, depending on institutional schedules. During this period, students are attached to government agencies, private industries, or organizations related to their field of study. They are expected to maintain logbooks, record daily activities, and produce reports summarizing their experiences at the end of the training.

SIWES also contributes significantly to national development by producing skilled manpower that can adapt to the evolving technological and industrial landscape of Nigeria. The Industrial Training Fund (ITF) oversees the program, working in collaboration with institutions of higher learning and employers of labor to ensure proper supervision and evaluation.

In essence, SIWES serves as a bridge between academic training and employment realities. It plays a vital role in equipping Nigerian students with practical knowledge, enhancing employability, and promoting self-reliance in a competitive labor market. The scheme has become an indispensable part of Nigeria's tertiary education system, preparing graduates to meet the challenges of national development and contribute effectively to the country's socio-economic growth.

1.2 OBJECTIVES OF SIWES

1. Provide avenues for students to acquire industrial skills and experience during their course of study.

2. Prepare students for industrial work situation they are likely to meet after graduation.
3. Expose students to work methods and techniques in handling equipment and machineries that may not be available in the university.
4. Provide students with the opportunities to apply their educational knowledge in real work situations, thereby bridging the gap between theory and practical.
5. To make the transition from the schooling to world of work easier through enhancing students' contact for later job placement.(ITF, 1973)

1.3 History and Background of Delta State Contributory Health Commission, Asaba

The Delta State Contributory Health Commission (DSCHC) was established by the Delta State Government as a statutory body through an enabling law passed by the Delta State House of Assembly and signed by the governor on 4 February 2016 to administer a state-wide health insurance scheme aimed at achieving universal health coverage for all residents of Delta State. The origin of the Commission began with the transmission of an executive bill in June 2015, which progressed through readings and public hearings before becoming law. This law created the Commission, the Delta State Contributory Health Scheme (DSCHS), and a governing board to regulate and implement a mandatory health insurance system across the state.

The DSCHC commenced operations on 1 January 2017, making Delta one of the first states in Nigeria to launch a structured contributory health insurance programme. Initial work included baseline assessments of household health behaviours, financial capacity, and health facility readiness to deliver services, guiding the design of benefit packages and implementation strategies.

Since inception, the Commission has grown its enrolment and provider network, enrolling millions of residents under various health plans—including formal, informal, equity, and private plans—and accrediting hundreds of public and private health facilities to deliver quality and affordable healthcare.

The DSCHC continues to evolve as a key healthcare financing institution in Delta State, partnering with public and private stakeholders to reduce out-of-pocket expenses and expand access to essential health services (DSCHC, 2016)

CHAPTER TWO

2.1 DELTA STATE CONTRIBUTORY HEALTH COMMISSION

The Delta State Contributory Health Commission is a government body established to manage and implement the Delta State Contributory Health Scheme. The scheme is a health insurance system that allows individuals and groups to contribute regularly toward the cost of healthcare services. Through this system, registered members are able to access a defined package of quality healthcare services whenever the need arises, without having to pay large sums of money at the point of care.

The main purpose of the contributory health scheme is to reduce the financial burden associated with healthcare expenses and to ensure that residents of Delta State can access healthcare services easily and affordably. Contributions made by enrollees are pooled together to create a sustainable fund that spreads health risks across the population. This approach helps protect individuals and families from unexpected medical costs and promotes continuous access to healthcare services, even during periods of financial difficulty.

The Commission is also responsible for accrediting and registering public and private healthcare facilities that meet the required standards for service delivery. These accredited facilities serve as access points for healthcare services, starting from primary healthcare centers and extending to secondary and tertiary facilities when referrals are required. In addition, the Commission ensures that vulnerable individuals who are unable to contribute are supported through funding from government and other partners, thereby promoting inclusive access to quality healthcare services for all residents of Delta State (DSCHC, 2016).



Fig 2.1 Delta State Contributory Health Commission Building